

Health Transformation Review (HTR) — User Manual

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1. What Is HTR?

Health Transformation Review (HTR) is a cross-disciplinary intelligence platform dedicated to health system transformation in the United States. HTR exists to answer a single question: *Why is changing the American healthcare system so hard — and what actually works?*

HTR is not a news site. It is not a think tank. It is a structured analytical platform that applies five distinct lenses — Policy, Economics, Technology, Clinical, and Equity — to every problem in healthcare. Each piece of intelligence published on HTR has been tested across all five dimensions before it reaches you. A policy that is legally permissible but financially unsustainable is not a solution. A technology that is technically possible but clinically ineffective is not progress.

Who uses HTR?

- **Hospital executives and administrators** tracking federal program funding, regulatory changes, and performance benchmarks
- **State health officials** monitoring performance across peer states
- **Policy analysts and legislative staff** tracking health legislation and regulatory activity
- **Healthcare investors and consultants** tracking capital flows, VBC adoption, and market dynamics
- **Clinical and public health professionals** tracking evidence-based care innovations
- **Equity advocates and community health leaders** tracking disparity data and SDOH integration
- **Graduate students and researchers** in health policy, public health, and health economics

The core promise

HTR tracks **50 states**, monitors **6,000+ hospitals and health systems**, and analyzes federal healthcare transformation programs and capital flows across the industry. Every piece of analysis is grounded in primary sources: CMS filings, legislative text, peer-reviewed evidence, and real financial disclosures. HTR does not trade in speculation dressed as insight.

2. The Five-Pillar Framework

HTR's analytical framework was originally built on three pillars (Policy, Economics, Technology) when HTR was founded in 2019. In 2024, Clinical and Equity pillars were formally added based on five years of evidence that clinical effectiveness and equity are not downstream considerations — they are foundational.

Each pillar asks a different essential question:

Policy — *Is it permissible?*

Color: Sky Blue

Policy analysis tracks the governance frameworks that determine what health system transformation is *allowed* to be. No health system initiative — no matter how promising clinically or economically — can succeed if it violates federal or state regulatory frameworks, conflicts with payer rules, or lacks legislative authority.

What HTR tracks under Policy:

- **Regulation & Legislation** — Federal rulemaking (CMS, FDA, FTC), state legislation, scope-of-practice laws, telehealth regulations
- **Public Health Mandates** — Emergency declarations, public health infrastructure requirements, reporting mandates
- **Global & Comparative Policy** — What other countries are doing and what the U.S. can learn
- **Policy Feasibility Studies** — Whether proposed policies can actually be implemented across different health system contexts

Economics — *Is it sustainable?*

Color: Emerald Green

Economic analysis tracks capital flows, workforce investment, and value-based care models. A clinical innovation that cannot be financed, a hospital that cannot survive on its reimbursement structure, or a workforce strategy that ignores labor market realities — these are not solutions.

What HTR tracks under Economics:

- **Value-Based Care Models** — APMs, ACOs, bundled payments, global budgets, AHEAD model, capitation
- **Market & Finance** — Hospital margins, merger & acquisition activity, payer market dynamics, insurance coverage rates
- **Labor & Workforce Strategy** — Physician and nursing supply/demand, workforce pipeline programs, international recruitment, compensation benchmarks
- **Healthcare Investment Trends** — Venture capital in rural health, federal grant programs, private equity behavior

Technology — *Is it possible?*

Color: Indigo

Technology analysis evaluates the digital infrastructure, AI systems, and data governance frameworks that amplify human clinical capacity. A technology that a hospital cannot afford to implement, cannot connect to their EHR, or cannot train their staff to use is not a usable technology.

What HTR tracks under Technology:

- **AI & Machine Learning** — Clinical AI tools, administrative AI, predictive analytics, ambient documentation
 - **Digital Health & Telemedicine** — Telehealth platforms, remote patient monitoring, digital therapeutics, asynchronous care
 - **Data Security & Governance** — HIPAA compliance, cybersecurity threats, data sharing agreements, interoperability (HL7 FHIR, USCDI)
 - **Tech-Enabled Workflow** — EHR optimization, care coordination platforms, health information exchange (HIE), broadband access
-

Clinical — *Is it effective?*

Color: Red

Clinical analysis bridges the gap between innovation and deployment reality at the point of care. Evidence generated at an academic medical center does not automatically translate to a 25-bed Critical Access Hospital. HTR tracks what actually works in resource-constrained environments.

What HTR tracks under Clinical:

- **Hospital-at-Home** — Acute-level home care programs, eligibility criteria, remote monitoring protocols
 - **Precision Medicine** — Genomic medicine access, pharmacogenomics, cancer precision diagnosis
 - **Virtual Care Models** — Synchronous and asynchronous telehealth, tele-ICU, telestroke, tele-psychiatry effectiveness data
 - **Population Health** — Preventive care penetration, chronic disease management (diabetes, cardiovascular, COPD), maternal health outcomes, readmission reduction
-

Equity — *Is it just?*

Color: Amber

Equity analysis holds transformation accountable to the communities it is meant to serve. Disparities by race, geography, income, and social circumstance are not marginal concerns — they are a foundational challenge of health system transformation. An innovation that widens disparities while improving average outcomes is not a success.

What HTR tracks under Equity:

- **SDOH Integration** — Social determinants screening, food and housing insecurity, transportation access in care delivery
 - **Algorithmic Bias** — Bias embedded in clinical AI tools, risk stratification algorithms, prior authorization systems
 - **Access Disparity** — Geographic, insurance, linguistic, and cultural barriers to care; uninsured/underinsured populations
 - **Community Engagement** — Community health workers, community benefit requirements, patient advisory councils
-

3. Getting Started — Navigation Overview

When you first open HTR, you see three main areas:

The Header (Top Bar)

The top of every page has a **dark bar** containing:

- The current date (desktop view)
- A **live news ticker** scrolling the latest policy, FDA, and CMS headlines
- A **FAQ** link (standalone, between the ticker and the COMPANY menu)
- A **COMPANY** dropdown with: About HTR, Mission & Vision, Core Values, Our Framework, Our Methodology, Contact Us

- A **Login** link
- A **Subscribe** button

Below the dark bar is the **main navigation bar** with:

- The **HTR logo** (returns to homepage)
- Five pillar dropdowns: **POLICY, ECONOMICS, TECHNOLOGY, CLINICAL, EQUITY**
- A **search bar** (type to search, or press **Cmd+K** / **Ctrl+K** on Windows)
- Sidebar toggle buttons (left and right edges)

The Left Sidebar

The left sidebar (visible on larger screens) gives you quick-access links organized into four labeled sections: **Services, Tools & Resources, Federal Programs,** and **State Initiatives**. See [Section 22](#) for the full structure. Click the sidebar toggle button (left side of header) to show or hide it.

The Right Sidebar

The right sidebar shows **AI Analyst** suggested questions — pre-written prompts you can click to instantly query the AI Analyst about current platform topics. Click any suggestion to open the full AI Analyst chat with that question pre-filled.

The Main Content Area

Everything between the sidebars is the main content area — articles, dashboards, academy modules, and more.

Keyboard Shortcuts

- **Cmd+K** (Mac) / **Ctrl+K** (Windows) — Open the command palette / search
- On mobile: use the hamburger menu (≡) to access navigation

4. The Homepage

The homepage gives you a high-altitude view of the platform:

- **Hero Carousel** — Featured analyses and breaking insights, rotating automatically
- **Pillar Overview** — Quick links into each of the five pillars with their core question
- **Latest from HTR** — Most recently published analyses across all pillars

From the homepage, navigate anywhere via the top nav pillars, the sidebar quick links, or the search bar.

5. The Pillar Hubs

Each pillar has its own **hub page** that you reach from the navigation dropdown. For example, clicking **POLICY** takes you to [/policy](#).

What a Hub Page Shows

Lead Story — The most recent or most important analysis in that pillar, displayed prominently with its impact level (Critical, High, or Medium).

Recent Briefs — A scrollable feed of the latest analyses showing category, publication date, title, and summary excerpt.

HTR Data Widget — A panel linking to the State Performance Dashboard for data behind this pillar.

Sub-Category Pages

Every pillar has 4–5 sub-categories accessible via the navigation dropdown:

Pillar	Sub-Categories
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Pillar	Sub-Categories
Policy	Regulation & Legislation, Public Health Mandates, Global & Comparative Policy, Policy Feasibility Studies
Economics	Value-Based Care Models, Market & Finance, Labor & Workforce Strategy, Healthcare Investment Trends
Technology	AI & Machine Learning, Digital Health & Telemedicine, Data Security & Governance, Tech-Enabled Workflow
Clinical	Hospital-at-Home, Precision Medicine, Virtual Care Models, Population Health
Equity	SDOH Integration, Algorithmic Bias, Access Disparity, Community Engagement

6. Reading Articles and Analysis

Article Header

- **Pillar badge** (color-coded by pillar)
- **Category** — the sub-category this analysis belongs to
- **Impact Level badge** — Critical (red pulsing), High (yellow), or Medium
- **Publication date** and **Author**

Article Body

HTR analyses are structured, not summary journalism. They typically include an executive summary, regulatory/policy context, data-driven analysis, cross-pillar implications, and actionable intelligence.

Article Toolbar

Every article page has:

- **Listen** — Text-to-speech reading
- **Print** — Print-formatted view
- **Save to PDF** — Download as PDF
- **Share** — Share via link or social
- **Font Size Toggle** — Adjust reading size

Related Articles

HTR recommends related analyses by pillar and category below each article.

7. The State Performance Dashboard

URL: /dashboard

The State Performance Dashboard tracks health system transformation progress across all 50 U.S. states using a composite scoring system across five pillars.

The National View

- **U.S. map** — States color-coded by performance status
- **State cards** — Each state listed with its performance score and status badge

Performance Status Labels

Status	Score Range	Meaning
Leading	80+	Outperforming national benchmarks. A model for peer learning.

Status	Score Range	Meaning
Improving	70–79	Meaningful upward trajectory. Active transformation programs producing measurable results.
Stable	50–69	At or near national average. Specific gaps exist but no acute crisis.
At Risk	Below 50	Multiple systemic failures. Urgent intervention required.

Drilling Into a State

Click any state card or map region to open the **State Detail Page** with up to three tabs:

1. **Performance Index** — The five-pillar scorecard with 15 metrics
2. **RHT Program** — Federal program profile (if the state participates)
3. **Hospital View** — Hospital-level data (if available)

Note: The State Performance Dashboard is distinct from the State Initiatives feature. The Dashboard tracks quantitative performance metrics and federal RHT program participation. State Initiatives (Section 19) is a separate feature covering health reform legislation, Medicaid waivers, and transformation programs across all 50 states.

8. Understanding the Performance Index Metrics

Each state's Performance Index tab shows **five pillar sections** with **three sub-metrics each** — 15 metrics total. Every metric is scored 0–100 where **100 = best possible performance**.

Score bar colors:

- **Green** (80+) — Strong performance
- **Light Green** (70–79) — Good performance
- **Yellow** (60–69) — Moderate, room for improvement
- **Red** (below 60) — Significant gap requiring attention

POLICY METRICS

VBP Adoption — Value-Based Payment Adoption

What it measures: The percentage of healthcare payments in the state made through value-based contracts (ACOs, bundled payments, global budgets, the CMS AHEAD model) rather than traditional fee-for-service billing.

Why it matters: Fee-for-service rewards volume — the more procedures, the more revenue. Value-based payment rewards outcomes — the better patients do, the more the provider earns. States with high VBP adoption have structurally realigned financial incentives toward prevention, care coordination, and access. States stuck in fee-for-service face a financial disincentive against the very investments that would improve health outcomes.

What a high score looks like: Vermont (90) — Vermont operates under an All-Payer Model where virtually all payers — commercial, Medicaid, Medicare — pay hospitals and providers through a global budget. Nearly 100% of hospital revenue flows through value-based contracts.

What a low score looks like: West Virginia (30) — The vast majority of payments remain fee-for-service. Providers are paid for each service rendered, with no financial incentive for keeping patients healthy between visits.

Telehealth Policy

What it measures: The strength of the state's telehealth regulatory framework — including Medicaid reimbursement parity for video and audio-only visits, cross-state practice permissions, originating site flexibility

(where patients can receive telehealth), and whether COVID-era telehealth flexibilities have been permanently retained.

Why it matters: Telehealth is the primary access mechanism for populations who may live 30–100 miles from the nearest specialist. Without payment parity — meaning insurers pay the same for a telehealth visit as an in-person visit — providers cannot financially sustain telehealth programs. States with strong telehealth policy have removed the regulatory and financial friction that prevents virtual care from scaling.

What a high score looks like: Massachusetts (90) — Full audio-video and audio-only parity across all payers, cross-state hub-and-spoke arrangements explicitly permitted, permanent retention of all COVID-era telehealth expansions, liberal originating site rules allowing patients to receive telehealth from home.

What a low score looks like: West Virginia (42) — Parity gaps between payers, restrictive originating site rules requiring patients to travel to a clinic to receive a telehealth visit, limited audio-only coverage constraining access for elderly patients without smartphones.

Scope of Practice

What it measures: How liberally the state allows non-physician clinicians to practice independently or with reduced physician oversight. This primarily covers Nurse Practitioners (NPs), Physician Assistants / Physician Associates (PAs), Certified Registered Nurse Anesthetists (CRNAs), and pharmacists.

Why it matters: America faces a physician shortage that cannot be closed by training more physicians alone — the pipeline is too slow and physicians overwhelmingly prefer urban settings. States with **full practice authority** for NPs allow these providers to act as primary care physicians in rural communities without a physician supervisor on-site. States with restrictive scope-of-practice laws that require physician oversight are effectively blocking the only realistically scalable solution to physician shortages in underserved areas.

States with full NP practice authority (high scores): Vermont, Massachusetts, California, Oregon, Washington, Minnesota — NPs can evaluate, diagnose, treat, and prescribe independently.

States with restricted practice (lower scores): Alabama, Georgia, South Carolina, Arkansas — NPs must have physician collaboration agreements and cannot open independent practices.

ECONOMICS METRICS

Spending Per Capita (Efficiency Score)

What it measures: An **inverted efficiency score** — a high score means **low per-capita healthcare spending relative to health outcomes achieved**. States that achieve strong health outcomes efficiently score higher. States with very high per-capita costs and poor outcomes score lower.

Important interpretation note: This is the one metric where the raw spending number is inverted. Always read this metric alongside the overall performance score and narrative to understand the full picture.

Why it matters: The goal of transformation is not just to spend more money — it is to achieve better outcomes at sustainable cost. States like Massachusetts achieve near-universal coverage and excellent outcomes at relatively efficient per-capita rates because their VBC models create system-wide cost discipline. States with high costs and poor outcomes have structural inefficiency that more money alone will not fix.

Workforce Gaps

What it measures: The degree to which the state's healthcare workforce supply meets the needs of its population. A score of 100 means near-complete coverage of primary care, mental health, dental, and specialty providers across the geography. Lower scores indicate significant **Health Professional Shortage Areas (HPSAs)** — federally designated regions where provider supply falls critically below the population need.

Why it matters: You cannot deliver healthcare without healthcare workers. A state can have perfect policy, abundant funding, and excellent technology infrastructure, but if there are no clinicians in rural counties, none of it

matters. States with the worst workforce scores — Kentucky (32), West Virginia (28), Alabama (28), Mississippi (35) — face a foundational access crisis that no technology or payment model can fully compensate for.

Key programs targeting this metric: Rural residency programs, loan forgiveness for rural practice, international medical graduate pathways, community health worker certification pipelines, high school-to-healthcare workforce programs.

Insurance Coverage Rate

What it measures: The percentage of the state population with health insurance coverage of any type — employer-sponsored, individual marketplace, Medicaid, Medicare, or other public coverage.

Why it matters: Uninsured and underinsured populations delay or avoid care, arriving at hospitals sicker and more expensive to treat. Uncompensated care — services provided to patients who cannot pay — is the primary driver of hospital financial distress. States that have not expanded Medicaid under the ACA have systematically higher uninsured rates, particularly in communities where employer-sponsored insurance is less common.

High coverage states: Massachusetts (97%), Vermont (95%), Hawaii (94%) — combination of Medicaid expansion, robust marketplace enrollment, and strong employer coverage.

Low coverage states: Texas (62%), Mississippi (58%), Oklahoma (60%) — non-Medicaid expansion states with high rates of uninsured residents.

TECHNOLOGY METRICS

HIE Adoption — Health Information Exchange Adoption

What it measures: The percentage of providers and hospitals in the state actively participating in a functioning Health Information Exchange — meaning they can electronically send, receive, find, and use clinical data across different organizations and EHR systems.

Why it matters: Patients frequently receive care from multiple providers across multiple counties, hospitals, and states. Without HIE, an emergency department treating a patient who had surgery two counties over has no access to that patient's surgical history, current medications, or allergies. HIE is the connective tissue of a coordinated health system. Without it, care coordination is based on what patients can remember to tell their providers — a dangerous foundation.

What full HIE looks like: Every provider in the state can query a shared network for any patient's medication list, recent lab results, imaging, discharge summaries, and care plans regardless of where that care was delivered.

Broadband Access

What it measures: The percentage of the state's underserved population with access to sufficient broadband internet — typically defined as 25 Mbps download / 3 Mbps upload or higher — to reliably support telehealth video visits, remote patient monitoring device data transmission, and digital health applications.

Why it matters: Every digital health strategy — telehealth, remote monitoring, AI-driven care tools, patient portals — requires reliable high-speed internet. Broadband infrastructure is a **healthcare infrastructure** problem, not merely a technology problem. States with severe broadband gaps have a hard floor under their entire digital transformation ambition.

States with the worst rural broadband scores: Alaska (42), West Virginia (35), Mississippi (45), Montana (52) — the most remote states predictably face the most severe broadband deficits, creating a compounding disadvantage where the states that need telehealth the most have the least ability to use it.

EHR Adoption — Electronic Health Record Adoption

What it measures: The percentage of hospitals and primary care practices in the state using a certified EHR system and meeting meaningful use / Promoting Interoperability criteria required by CMS.

Why it matters: EHRs are the foundational data infrastructure of modern healthcare. Population health analytics, quality reporting, value-based contract performance measurement, care gap identification, and all clinical AI tool integration depend on structured electronic data. States with low EHR adoption cannot participate in modern care delivery models, cannot measure their own performance, and cannot leverage digital tools that require structured data as inputs.

CLINICAL METRICS

Preventive Care

What it measures: The percentage of eligible patients receiving recommended preventive care services — including cancer screenings (mammography, colonoscopy, cervical cancer, lung CT), immunizations (influenza, pneumococcal, COVID), well-child visits, annual wellness visits, preventive dental care, and behavioral health screenings.

Why it matters: Preventive care is where chronic disease and cancer are stopped before they start, or caught early when they are most treatable. Populations with access barriers (no local provider, transportation costs, appointment availability) have historically low preventive care utilization. A state that screens 85% of eligible women for cervical cancer will have dramatically lower late-stage cervical cancer mortality than one that screens 55%. The difference is not genetic — it is infrastructure.

Readmission Rate (Score)

What it measures: An inverted 30-day hospital readmission score — a **high score means low readmission rates**. This captures 30-day all-cause readmissions for the most common conditions: heart failure, pneumonia, COPD, hip/knee replacement, and acute myocardial infarction (heart attack). Lower real-world readmission rates produce a higher score.

Why it matters: Hospital readmission is one of the most reliable indicators of care coordination failure. When a patient is discharged and returns to the hospital within 30 days for the same or related condition, it almost universally reflects a breakdown in post-discharge support — medication management, follow-up appointment access, transportation to follow-up care, social support, or patient education. High readmission rates harm patients and are financially penalized by CMS under the Hospital Readmissions Reduction Program (HRRP).

Chronic Disease Control

What it measures: The percentage of patients with diagnosed chronic conditions — primarily Type 2 diabetes, hypertension, heart failure, COPD, and asthma — whose condition is under clinical control. For diabetes: HbA1c under 8.0%. For hypertension: blood pressure under 140/90 mmHg.

Why it matters: Chronic disease is the dominant burden of American health and the primary driver of hospital utilization and financial distress. Uncontrolled diabetes leads to dialysis, lower extremity amputation, blindness, and early death — all of which are expensive to treat and preventable with adequate chronic disease management. States with high chronic disease control scores have invested in the care management infrastructure — patient coaching, care navigators, community health workers, medication adherence programs — that demonstrably reduces catastrophic downstream costs.

EQUITY METRICS

Racial Equity Gap

What it measures: The inverse of the measured disparity in health outcomes between racial/ethnic groups within the state. A score of 100 means no measurable disparity between white and non-white populations on key

outcomes — maternal mortality, infant mortality, diabetes control, cancer survival rates, life expectancy, preventive care utilization. Lower scores indicate larger outcome gaps.

Why it matters: The existence of racial gaps in health outcomes is not biological destiny. It is the result of documented structural underinvestment, discriminatory policy history, and ongoing social determinant disparities. States with the largest racial gaps — Alabama (20), Mississippi (22), Louisiana (20), Georgia (28) — carry the deepest legacy of structural exclusion that health transformation must deliberately address and measure.

What improvement looks like: Targeted maternal health programs for Black women, community health worker deployment in high-disparity zip codes, SDOH screening and navigation in communities of color, cultural competency training, language access, and deliberate provider recruitment into underserved communities.

Rural-Urban Gap

What it measures: The disparity in health outcomes and access between rural and urban populations within the same state. A score of 100 means rural residents have equivalent access and outcomes to urban residents. A low score indicates rural residents are substantially worse off than urban counterparts in the same state.

Why it matters: Many states have strong overall health metrics driven entirely by their major urban centers — Boston, New York, Chicago — while rural populations in the same state face care deserts, high mortality, and no specialist access. The rural-urban gap score strips away the urban averaging effect to reveal the true state of rural health within each state.

SDOH Integration — Social Determinants of Health Integration

What it measures: The degree to which the state's healthcare delivery system has embedded Social Determinants of Health (SDOH) screening and response into routine clinical care. Specifically: food insecurity screening at primary care visits, housing instability navigation programs, transportation assistance for medical appointments, community health worker deployment at scale, and systematic linkage to social services following SDOH screening.

Why it matters: Research consistently shows that 60–80% of health outcomes are driven by factors outside the clinical encounter — poverty, housing stability, food security, education level, transportation access, social isolation, and neighborhood safety. A provider who treats a diabetic patient's blood sugar but does not address the food desert the patient lives in is managing a symptom without treating a cause.

What high SDOH integration looks like: Every primary care visit includes a validated social needs screening tool (like the PRAPARE protocol). Positive screens trigger warm handoffs to community health workers who connect patients to food banks, housing navigators, transportation vouchers, and utility assistance.

The State Narrative

Each state detail page includes a **narrative section** — a headline and analytical summary written by HTR analysts providing qualitative context to the scores. This explains *why* a state is performing as it is, what specific programs are driving change, and what the key strategic challenges are. Read the narrative alongside the scores to understand the full picture.

9. The RHT Program Tab

The **Rural Health Transformation (RHT) Program** is a federal initiative through which participating states receive substantial federal grants — ranging from tens of millions to over \$1 billion — to redesign their healthcare systems. The RHT Program tab shows each state's specific program profile.

What you see in the RHT Tab

Award Amount — The total federal grant commitment displayed in the state header.

Program Description — The state's strategic theory of change — what transformation they are attempting and why.

Strategic Initiatives — The specific programs funded by RHT dollars. Each initiative has a name and description. Examples:

- "Lone Star AI Telehealth Network" (Texas) — AI-driven telehealth hubs serving rural counties
- "PoWERing Maternal Health" (Kentucky) — Full year of postpartum support for high-risk mothers
- "ROOTS Regional Hubs" (North Carolina) — Six regional integration hubs breaking down medical/social silos
- "Rural Center of Excellence" (Montana) — Centralized oversight coordinating statewide restructuring

Key Metrics — Specific performance commitments with their current status:

- ✓ **Achieved** (green) — Target met
- 🕒 **In Progress** (amber) — Working toward target
- ⚠️ **Not Met** (red) — Behind target

How to use the RHT Tab

The Performance Index tells you *where a state is*. The RHT Program tab tells you *what they are actively doing about it*. Together, they show both the diagnosis and the treatment plan — and whether the treatment is working based on metric status.

10. The Hospital View Tab

The **Hospital View** tab shows performance data for individual hospitals within the selected state.

Column Definitions

Column	Definition
Hospital	Full hospital name
City	Location city
Type	Critical Access Hospital (CAH), Rural PPS, or Urban
Discharges	Total annual inpatient discharges — a volume measure
Avg. Stay	Average inpatient length of stay in days
Quality Score	Composite quality performance score (0–100), color-coded

Sorting

Click any column header to sort. Click again to reverse. Arrows indicate current sort direction.

Understanding Hospital Types

Critical Access Hospital (CAH): A Medicare designation for rural hospitals with ≤25 beds that are geographically isolated as the sole community provider. CAHs receive cost-based Medicare reimbursement rather than DRG rates, protecting their financial viability. There are approximately 1,366 CAHs nationally — they are often the most financially fragile and most essential institutions in rural health.

Rural PPS Hospital: A rural hospital paid through the standard Prospective Payment System (DRG-based rates) rather than cost-based reimbursement. Generally larger than CAHs.

Urban: Urban hospitals included in the state view for comparison context.

11. The AI Analyst (Chat)

URL: </chat>

The HTR AI Analyst is powered by Google Gemini enhanced with **Retrieval-Augmented Generation (RAG)**. When you ask a question, the system first searches HTR's entire content library (articles, analyses, academy modules, glossary, case studies, analyst notes, and primary source documents) for relevant information, then generates an answer grounded in that specific content.

This means the AI Analyst is not a generic chatbot — it has deep knowledge of HTR's specific analyses and can cite platform content in its answers.

How to Use It

- 1. Navigate to [/chat](#) or click **"Open Full Chat"** in the right sidebar
- 2. Type your question in the input box
- 3. Press Enter or click Send
- 4. The AI streams its response in real time
- 5. Ask follow-up questions — the AI maintains the full context of your conversation

What the AI Analyst Does Well

- **Policy explanations** — "What is the CMS AHEAD model and which states are participating?"
- **State comparisons** — "How does Minnesota's approach to mental health differ from Tennessee's?"
- **Metric explanations** — "What does the Scope of Practice score measure and why does it matter?"
- **Content synthesis** — "What are HTR's key findings on SDOH integration?"
- **Glossary queries** — "What is value-based care?" / "Define FQHC."
- **Program details** — "What is the Rural Health Transformation program?"
- **Cross-pillar reasoning** — "What are the economic implications of Georgia's obstetric crisis?"
- **State initiative context** — "What is Vermont Act 167 and why does it matter?"
- **Document analysis** — "What did the Oliver Wyman Report find about Vermont hospital transformation?"

What It Cannot Do

- Access real-time internet data beyond the HTR knowledge base
- Provide specific legal or medical advice for your situation
- Make financial projections or forecasts
- Access information not in the HTR content library

Tips for Better Queries

Be specific: *"What is Vermont's VBP adoption rate and why is it so high?"* is better than *"How is Vermont doing?"*

Ask for explanations: *"Explain what HIE adoption means and why low scores hurt rural hospitals."*

Ask for implications: *"What does West Virginia's 31 performance score mean strategically for a hospital executive in that state?"*

Ask comparatively: *"Which At Risk states are showing the most improvement and what programs are driving it?"*

Ask about initiatives: *"What does California's CalAIM program do for housing-insecure patients?"*

Using the Right Sidebar Suggestions

The right sidebar shows three pre-written suggested questions reflecting current platform topics. Clicking any suggestion opens the AI Analyst with that question ready to submit.

12. The HTR Academy

URL: [/academy](#)

The HTR Academy provides structured educational content for building deep foundational knowledge in healthcare transformation — distinct from the analysis hubs, which publish ongoing intelligence.

Content Types

Modules

Standalone learning units (15–90 minutes each) on specific topics. Each module includes:

- **Learning objectives** — What you will know and be able to do after completing it
- **Structured content** — Formatted with headings, key concepts, data, and case examples
- **Duration** — Time estimate in minutes
- **Pillar tag** — Which of the five pillars this belongs to
- **Course navigation** — Previous and next module links when part of a sequence

Courses

Multi-module learning sequences organized around a coherent subject. Course formats:

- **CERTIFICATION** — Credentialed completion programs
- **COURSE** — Structured multi-module sequences
- **WEBINAR** — Live or recorded expert sessions
- **MASTERCLASS** — Premium in-depth executive programs

Each course page shows the module sequence, instructors, format type, and price (if applicable).

Webinars

Live and recorded sessions from HTR faculty and guest experts, tagged by pillar.

Case Studies

Real-world implementation stories from hospitals, states, and health systems. Each case study shows the client type, executive summary, key metrics achieved, and the full narrative.

Faculty

The [/academy/faculty](#) page lists all HTR Academy instructors with their credentials, roles, and associated courses.

Navigating Within a Module

- Module header shows title, module number (e.g., "Module 2 of 5"), course name, and duration
- Learning objectives appear at the top
- Previous / Next navigation buttons at the bottom move through the course sequence

13. The Glossary

URL: [/academy/glossary](#)

The HTR Glossary decodes the specialized lexicon of healthcare transformation — federal program names, clinical terms, regulatory concepts, and policy jargon.

Finding Terms

Search: Type any keyword in the search box. Results filter in real time against both term names and definitions.

Pillar filter: Click any pillar button to show only terms relevant to that pillar. Each pillar tag is color-coded:

- Policy (sky blue), Economics (emerald green), Technology (indigo), Clinical (red), Equity (amber)

Alphabet bar: Click any letter to show terms beginning with that letter. Dimmed letters indicate no terms are available.

Pillar Badges

Each glossary term may carry one or more pillar badges showing which analytical frameworks the term belongs to. A term like "Value-Based Care" may be tagged as both Economics and Policy. "SDOH" is tagged as Equity. "EHR" is tagged as Technology.

14. Search

URL: </search> | Keyboard shortcut: **Cmd+K** / **Ctrl+K**

Search covers six content types simultaneously:

- Articles and posts
- Policy analyses
- Academy modules
- Glossary definitions
- Case studies
- Analyst notes (active only)

Type at least 2 characters. Results appear grouped by content type with title, summary, and a direct link.

15. Multimedia — Videos and Audio

URL: </multimedia>

Video Library

Video briefings are organized by pillar in colored sections (Policy, Economics, Technology, Clinical, Equity). Within each pillar section:

- **Category filter buttons** — Filter videos by sub-category
- **Global search bar** — Search across all videos
- **Load More** — Reveal additional videos in a section
- Each tile shows the video player and a linked article title

Audio

Some articles include embedded audio briefings in the article body. The **Listen** button on any article toolbar also activates text-to-speech reading.

16. The Advisory Hub

URL: </advisory>

HTR's professional services arm for organizations that need more than published intelligence:

- **Strategic Consulting** — Engagements for hospital executives, state officials, and investors
- **Custom Research** — Bespoke reports on topics not covered by standard analysis
- **Annual Reports** — Comprehensive annual state/regional assessments
- **Contact** — Direct inquiry form at </advisory/contact>

17. Trending Topics

URL: </trending-topics>

Surfaces the most actively discussed topics across HTR's content — topics with the most recent publication activity and highest AI Analyst query volume. Use Trending Topics to quickly identify what is urgent in healthcare transformation right now and find areas you may not have been tracking.

18. Federal Programs — AHEAD Model & Rural Health Transformation

HTR maintains dedicated analysis pages for the two major federal healthcare transformation programs currently active.

Rural Health Transformation (RHT) Program

URL: </dashboard>

The RHT Program (accessible via the Performance Dashboard) is the CMS Innovation Center's flagship rural health initiative. Participating states receive multi-year federal grants to redesign their healthcare delivery systems. The dashboard shows:

- All participating states and their award amounts
- Program status (Active, Pending, At Risk)
- Strategic initiatives funded by RHT dollars
- Performance metrics and their current achievement status

Navigate directly to any state's RHT profile via the dashboard map or the **Rural Health Transformation** link in the left sidebar.

AHEAD Model

URL: </ahead-model>

The **AHEAD Model** (Advancing All-Payer Health Equity Approaches and Development) is a 10-year CMS federal program (2024–2034) designed to align all payers — Medicare, Medicaid, and commercial insurers — around shared total cost of care targets and health equity metrics.

What the AHEAD page covers:

- Program structure and design principles
- The six participating states: Connecticut, Hawaii, Maryland, Minnesota, New Hampshire, and Vermont
- How AHEAD builds on the Vermont All-Payer Model experience
- Global budget mechanics and equity accountability requirements
- What AHEAD means for hospital strategy and state health policy
- Connection to Act 167 in Vermont and CalAIM in California

Who should read it: Hospital CFOs, state budget officers, ACO executives, payer strategy teams, and any organization operating in an AHEAD state needs to understand how this program will restructure their revenue environment through 2034.

19. State Initiatives — Vermont, California, and All 50 States

HTR tracks health reform legislation, Medicaid waivers, and system transformation programs at the state level. This section covers three levels of depth: deep-dive pages for Vermont and California, and the 50-state initiatives map.

Vermont Act 167

URL: </vermont-act-167>

Vermont Act 167 (2023) is one of the most significant pieces of state health legislation enacted in the United States in recent decades. It restructures how Vermont funds and operates its hospital system and represents a direct application of the conclusions from the Oliver Wyman Report on Vermont hospital transformation.

What the Vermont Act 167 page covers:

The Oliver Wyman Report — The landmark 2023 analysis commissioned by Vermont's Green Mountain Care Board that found Vermont's hospital network was financially unsustainable in its current configuration and recommended consolidation from 14 hospitals to a coordinated network of specialty-appropriate facilities.

Act 167 Provisions — The law's key requirements:

- Mandatory statewide hospital needs assessment

- Expanded Green Mountain Care Board (GMCB) authority over hospital budgets and capital expenditures
- Required development of a 10-year transformation plan
- New oversight powers for hospital mergers and service line changes
- Enhanced reporting requirements for all hospital financial operations

All-Payer Model Context — How Act 167 fits within Vermont's existing All-Payer Model agreement with CMS, and how it positions Vermont for the AHEAD Model's next phase.

Policy Implications — What Vermont's approach means for other states considering similar hospital governance legislation. Vermont is the first state to give its health oversight board this level of authority over hospital structure.

Who should read it: Vermont hospital executives and boards, New England health system strategists, state health policymakers nationwide considering hospital consolidation legislation, health lawyers tracking GMCB authority expansion.

California CalAIM

URL: </california-calaim>

CalAIM (California Advancing and Innovating Medi-Cal) is California's \$6.7 billion Section 1115 Medicaid demonstration waiver — the largest Medicaid transformation initiative in the United States by dollar value. It fundamentally restructures how California's 15 million Medi-Cal (Medicaid) enrollees receive care by integrating medical, behavioral health, and social services under a whole-person care framework.

What the California CalAIM page covers:

Enhanced Care Management (ECM) — Intensive care coordination for the highest-need, highest-cost Medi-Cal members: people experiencing homelessness, justice-involved individuals, people with complex behavioral health conditions, and those with multiple chronic conditions. ECM assigns a dedicated care manager who coordinates all services including medical, behavioral, and social needs.

Community Supports — A menu of 14 social service benefits that Medi-Cal managed care plans can now pay for, including:

- Medically Tailored Meals (post-discharge nutrition support)
- Housing Transition Navigation
- Housing Deposits
- Short-term Post-Institutionalization Stays
- Transportation (non-emergency medical and broader)
- Recuperative Care (medical respite for unhoused individuals)
- Day Habilitation Programs

Population Health Management (PHM) — Requirement for all Medi-Cal managed care plans to segment their populations by risk, proactively identify gaps in care, and develop systematic outreach strategies for preventive care, chronic disease management, and behavioral health.

Dental Transformation — Major expansion of Medi-Cal dental benefits and provider payment rates, with a focus on preventive care and elimination of access barriers.

Equity-Centered Design — CalAIM's accountability framework explicitly requires managed care plans to track and close disparities by race, ethnicity, language, and geography.

Statewide Rollout Timeline — How CalAIM is being implemented across California's 58 counties and 24 managed care plans from 2022 through 2027.

Who should read it: Medicaid managed care executives, community health organizations, housing navigators and social service providers, behavioral health organizations, health equity practitioners, and any organization that works with Medi-Cal populations anywhere in California.

Other States — The 50-State Initiatives Map

URL: </states>

The Other States page provides a comprehensive interactive browser of health reform initiatives across all 50 U.S. states and the District of Columbia. This is the broadest view of what American states are actually doing to transform their health systems.

The Interactive Map

The US map at the top of the page colors each state based on the number of active health reform initiatives it has in HTR's database:

- **1 initiative** — light orange
- **2 initiatives** — medium orange
- **3 initiatives** — darker orange
- **4 or more initiatives** — deep orange

Click any state on the map to navigate directly to that state's initiative profile.

Search and Filter

Below the map:

- **Search bar** — Filter states by name or by initiative type/keyword in real time
- **Region filter buttons** — Filter to show only states in a specific U.S. Census region: Northeast, Midwest, South, or West

State Cards

Each state card shows the state name, region, and number of tracked initiatives. Click any card to open the full state profile.

Individual State Pages ([/states/\[state\]](/states/[state]))

Each state's profile page shows:

State hero — State name, region, and abbreviation

Initiative cards — Each initiative HTR tracks for that state includes:

- **Title** — The initiative name (e.g., "Oregon Medicaid 1115 Waiver Renewal," "Minnesota Public Option Proposal")
- **Type badge** — The category of reform:
 - *Medicaid Waiver* — Section 1115, DSRIP, or other CMS demonstration waivers
 - *State Legislation* — Laws enacted by the state legislature
 - *Federal Program* — Federal grants or programs the state participates in (RHT, AHEAD, etc.)
 - *Executive Action* — Governor's orders or agency rulemakings
 - *Regional Compact* — Multi-state agreements and compacts
- **Status badge:**
 - *Active* (green) — Currently in operation
 - *Proposed* (amber) — Under legislative or regulatory consideration
 - *Completed* (gray) — Successfully concluded
 - *Under Review* (blue) — Awaiting CMS or legislative approval
- **Year** — When the initiative was enacted or proposed
- **Description** — What the initiative does and why it matters
- **View Source** link — Direct link to the official government or CMS source document

States with Featured Deep-Dive Pages

For states where HTR has published comprehensive analysis pages, the state profile shows a banner linking to that deeper coverage:

State	State Profile	Featured Page
Vermont	/states/vermont	/vermont-act-167 — Act 167 + Wyman Report deep dive
California	/states/california	/california-calaim — CalAIM \$6.7B waiver deep dive

Additional deep-dive pages are planned for Maryland (Total Cost of Care model), Maine (primary care transformation), Minnesota (public option), and Pennsylvania (rural hospital consolidation).

What the 50-State Map Covers

HTR's state initiatives database includes 130+ tracked initiatives across every state. Types covered include:

- **Medicaid expansion and waivers** — Section 1115 demonstrations, managed care delivery system reforms, HRSN (health-related social needs) waivers
- **All-payer and global budget models** — Maryland's TCOC model, Vermont's All-Payer Model, AHEAD states
- **Prescription drug pricing** — State importation programs, drug pricing transparency laws, insulin pricing caps
- **Behavioral health integration** — State 988 implementation plans, mental health parity enforcement, SUD treatment reforms
- **Workforce** — State-funded medical school expansions, loan forgiveness programs, scope-of-practice legislation updates
- **Hospital finance reform** — Global budget pilots, closure moratoriums, Certificate of Need (CON) reform
- **Public option and insurance market reform** — State-based public option programs, reinsurance arrangements, market stability measures
- **Health equity mandates** — Disparity reporting requirements, community benefit rules, targeted maternal health programs

20. Creating an Account

URL: </signup>

HTR accounts provide:

- Enrollment in the weekly intelligence digest
- Access to subscriber-only content and article comments
- Account management at </account>

Sign Up Process

1. Navigate to </signup>
2. Enter your email and password
3. Click **Create Account**
4. Confirm your email via the link sent to your inbox
5. You are automatically redirected to the dashboard and subscribed to the digest

Login / Password Reset

- Login at </login>
- Forgot password: click **Forgot password?** → enter email → use the reset link sent to your inbox → set new password at </reset-password>

21. The Newsletter Digest

A weekly intelligence email automatically sent to all active subscribers containing:

- The 5 most recent policy analyses
- Direct links to full articles
- Platform highlights

Manage your subscription and digest preferences at [/account](#) after logging in.

22. The Left and Right Sidebars

Sidebars are **context-sensitive** — visible on browsing pages, automatically hidden on focused reading pages (articles, academy modules, chat).

Left Sidebar — Quick Navigation

The left sidebar is organized into **four labeled sections** with color-coded pill badges and white cards with colored left-border stripes.

Services *(indigo)*

Quick access to HTR's two professional service platforms:

- **Academy** ([/academy](#)) — Structured educational courses, modules, webinars, and case studies
- **Advisory** ([/advisory-hub](#)) — Strategic consulting and custom research services

Tools & Resources *(amber)*

Quick access to HTR's analytical and research tools:

- **Research Lab** ([/hti-dashboard](#)) — The Health Transformation Index dashboard with cross-state analytical tools
- **Multimedia** ([/multimedia](#)) — Video briefings and audio content organized by pillar
- **Trending Topics** ([/trending-topics](#)) — Most active topics across HTR's content right now

Federal Programs *(emerald)*

Direct links to HTR's federal program tracking:

- **Rural Health Transformation** ([/dashboard](#)) — The 50-state RHT program dashboard with interactive map, state-by-state program profiles, performance metrics, and hospital data
- **AHEAD Model** ([/ahead-model](#)) — The CMS 10-year all-payer model: program design, participating states, global budget mechanics, and strategic implications

State Initiatives *(rose)*

Direct links to HTR's state-level health reform intelligence:

- **Vermont Act 167** ([/vermont-act-167](#)) — Vermont's hospital transformation law + Oliver Wyman Report analysis
- **California CalAIM** ([/california-calaim](#)) — California's \$6.7B Medi-Cal transformation waiver deep dive
- **Other States** ([/states](#)) — Interactive 50-state map with health reform initiatives across every U.S. state

Right Sidebar — AI Analyst Entry Point

Three rotating suggested questions. Clicking any opens the AI Analyst with that question pre-filled. **"Open Full Chat"** navigates directly to [/chat](#).

Toggle Controls

- Left sidebar: Menu icon on the left side of the navigation bar
- Right sidebar: Panel icon on the right side of the navigation bar
- Auto-hidden below 1024px (left) and 1280px (right) screen width

23. The News Ticker

A continuously scrolling strip in the dark top bar showing live headlines from three sources:

Label	Source
Policy	KFF Health News
FDA	Google News — FDA approvals and regulatory actions (last 48 hours)
CMS	Google News — Medicare and Medicaid news (last 48 hours)

Headlines refresh every 5 minutes. Each headline is clickable and opens the source article.

24. Frequently Asked Questions

Q: What is the difference between the Performance Index and the RHT Program?

A: The Performance Index is HTR's own composite scoring system measuring where a state currently stands across 15 metrics in five pillars. The RHT Program tab shows the actual federal Rural Health Transformation grant program — which states participate, how much money they receive, and what specific programs they are funding with it. A state can score high on the Performance Index without being in the RHT Program (they were already performing well), and a state can be in the RHT Program with a low index score (they are attempting to improve from a troubled baseline).

Q: What is the difference between the Performance Dashboard and the State Initiatives map?

A: The **Performance Dashboard** ([/dashboard](#)) tracks quantitative metrics — scoring each state across 15 measures in five pillars — and the federal RHT grant program participation. It answers: *How well is this state performing?* The **State Initiatives map** ([/states](#)) is a qualitative intelligence tool that catalogs what states are actually doing: laws they have passed, waivers they have filed, and federal programs they participate in. It answers: *What is this state trying to do?* The two tools are complementary — use the dashboard to diagnose performance, use the initiatives map to understand strategy.

Q: Are these scores official government data?

A: The HTR Performance Index scores are HTR's own composite analytical scores built from publicly available sources (CMS quality data, Census health surveys, HRSA workforce data, FCC broadband maps, state health department reports). They are not official government ratings. The RHT Program data reflects the actual federal program. See [/about/methodology](#) for full methodology documentation.

Q: How often is the data updated?

A: Performance Index metrics update on a rolling basis as new government data releases occur — typically annually for most metrics, quarterly for a few. News ticker data refreshes every 5 minutes. RHT Program data updates when federal program status changes. State initiative data updates as HTR analysts track new legislation and waiver filings.

Q: What does the AI Analyst know?

A: The AI Analyst has access to all content published on HTR — articles, analyses, academy modules, glossary definitions, case studies, and analyst notes — as well as primary source research documents including the Wyman Report on Vermont hospital transformation and Vermont Act 167. It also retains the context of your current conversation, so follow-up questions work naturally. It does not have real-time internet access, does not know your personal account data, and does not have information from outside the HTR knowledge base.

Q: Why do some At Risk states have large RHT Program awards?

A: The federal RHT program specifically targets states that are struggling most and need transformation investment. An At Risk state with large RHT funding is one where the federal government has recognized the crisis

and committed to a solution. Whether that investment produces measurable improvement over time is exactly what the Performance Index tracks.

Q: What is a Critical Access Hospital?

A: A Medicare designation for rural hospitals with ≤25 beds that are geographically isolated as their community's sole acute care provider. CAHs receive cost-based Medicare reimbursement rather than standard DRG rates to protect their financial viability. There are approximately 1,366 CAHs nationally — they are the backbone of rural acute care and the most financially vulnerable institutions in the health system.

Q: Where do I find HTR's Core Values?

A: Core Values have their own dedicated page at [/values](#), also accessible via the COMPANY menu. They are also prominently featured on the Mission page ([/mission](#)).

Q: Where is the Five-Question Standard (Our Framework)?

A: The Five-Question Standard is on the dedicated page at [/about/framework](#), accessible via the COMPANY menu under "Our Framework." It explains how HTR applies all five pillars to every analysis before publication.

Q: What is the AHEAD Model?

A: The AHEAD Model (Advancing All-Payer Health Equity Approaches and Development) is a CMS federal program running through 2034, active in 6 states (CT, HI, MD, MN, NH, VT). See [/ahead-model](#) for the full program profile — including how it builds on Vermont's All-Payer Model, what global budget commitments it requires, and what it means for hospitals in participating states.

Q: What is Vermont Act 167 and why is it important?

A: Vermont Act 167 (2023) is a state law that dramatically expands the Vermont Green Mountain Care Board's authority to oversee hospital budgets, capital spending, and system restructuring. It was passed in direct response to the Oliver Wyman Report finding that Vermont's hospital network was financially unsustainable. It is nationally significant because it is the first time a state has given its health oversight body this level of structural authority over hospital operations. See [/vermont-act-167](#) for HTR's full analysis.

Q: What is CalAIM?

A: CalAIM (California Advancing and Innovating Medi-Cal) is California's \$6.7 billion Section 1115 Medicaid waiver, the largest Medicaid transformation program in the country by dollar value. It integrates medical, behavioral health, and social services for California's 15 million Medi-Cal enrollees and creates new benefits for housing support, food security, and transportation. See [/california-calaim](#) for HTR's full analysis.

Q: How do I find what my state is doing on health reform?

A: Navigate to [/states](#) for the interactive 50-state map, then click your state to see all tracked initiatives. You can also filter by region or search by keyword. For Vermont and California, dedicated deep-dive pages at [/vermont-act-167](#) and [/california-calaim](#) provide comprehensive analysis.

Q: How do I report a data error or platform issue?

A: Use the contact form at [/advisory/contact](#). Include the specific page URL and what you believe is incorrect.

Q: Can I download data?

A: Individual articles can be saved as PDF using the toolbar button. Bulk data export is on the platform roadmap. For custom data needs, contact the Advisory team via [/advisory/contact](#).

End of HTR User Manual — Version 2026 (updated 2026-03-14)

For technical documentation, architecture details, and developer reference, see DEVELOPER_GUIDE.md